

Women’s Health Requirements Discovery Pack

Colposcopy and Gynaecology Referral Grading

Prepared from the email provided by Dr Jasveen Kaur and the three attached reference files. Generated on 2026-03-23.

Purpose

This handout is intended for a requirement-gathering session. It is prefilled with current understanding so the clinical, operational, and technical stakeholders can confirm, correct, or extend the assumptions.

How To Use

1. Review the assumptions in order. 2. Mark each item as confirmed, changed, unknown, or out of scope. 3. Record the owner for each unresolved area. 4. Use the final decisions sheet to identify who will validate colposcopy, gynaecology, workflow, and technical integration questions.

Included Sections

1. Current understanding • 2. Clinical validation • 3. Workflow and operations • 4. Technical, integration, AI, and hosting • 5. Decisions and owners

1. Current Understanding

01-current-understanding.csv

Area	Requirement or understanding	Source	Current assumption	Status	Why this matters	Please confirm or correct	Notes
Problem	Gynaecology grading is difficult because it requires reviewing multiple sources such as clinic letters, discharge summaries, lab results, and radiology.	Email from Jasveen	Yes, this is a core pain point.	Assumed	This drives the need for a document-first workflow and summary view.		
Problem	The first useful step is a one-page summary to reduce clicks for the grader.	Email from Jasveen	Yes, this appears to be the immediate practical need before full automation.	Assumed	This defines the first product outcome even before autonomous AI grading.		
Problem	The longer-term goal is for the system to learn from clinician grading and automate most grading over time.	Email from Jasveen	Yes, but this should likely come after a validated clinician-in-the-loop phase.	Assumed	This affects whether phase 1 is deterministic, assistive AI, or future autonomous AI.		
Problem	Colposcopy may be easier to structure first because it is more category-based and requires less breadth of supporting information.	Email from Jasveen	Yes, colposcopy looks like the simpler first validation candidate.	Assumed	This affects pilot sequencing and project risk.		
Governance	Data sovereignty is important and may limit where data and AI processing can be hosted.	Email from Jasveen	Yes, likely NZ-hosted or otherwise approved Health NZ-controlled environment.	Assumed	This affects hosting, AI platform, storage, and integration choices.		
Governance	Access to the national colposcopy database is restricted and requires confidentiality and safety training.	Email from Jasveen	Yes, access appears to be controlled by training and governance requirements.	Assumed	This affects colposcopy accuracy, user access control, and pilot feasibility.		
Colposcopy	Colposcopy grading should align with the attached colposcopy grading guide and current grading template.	COLP Grading Guide + Current colposcopy grading template	Yes, the operational workflow should match existing service practice rather than a generic triage model.	Assumed	This determines what fields, notes, and outputs the colposcopy workflow needs.		
Colposcopy	Colposcopy requires less supporting information than gynaecology, but may still need national registry context for accuracy.	Email from Jasveen	Yes, some colposcopy cases may depend on prior national registry history.	Assumed	This affects whether colposcopy can be piloted without live registry integration.		
Gynaecology	Gynaecology grading should follow the attached guideline categories and priority rules.	Gynaecology guideline PDF	Yes, the guideline is the starting point, but local service interpretation may still vary.	Assumed	This affects category-specific decision logic and workflow branching.		
Gynaecology	AUB grading likely depends on ultrasound availability, symptoms, and management already tried.	Gynaecology guideline PDF	Yes, this appears to be a structured evidence-dependent category.	Assumed	This affects mandatory data capture and reject or return-to-GP rules.		
Gynaecology	PMB grading likely depends on ultrasound and endometrial thickness or similar uterine findings.	Gynaecology guideline PDF	Yes, PMB appears to be a key high-priority category with threshold-based logic.	Assumed	This affects urgency and whether Rapid Access or HSC pathways are needed.		
Gynaecology	Fibroid grading likely depends on size plus symptoms such as bleeding, pressure, or urinary effects.	Gynaecology guideline PDF	Yes, fibroid referrals appear to be graded by size and symptom burden rather than diagnosis alone.	Assumed	This affects what evidence is mandatory for triage.		
Gynaecology	Ovarian or adnexal mass referrals likely depend on suspicious imaging features and tumour markers such as CA-125.	Gynaecology guideline PDF	Yes, this appears to be an HSC-sensitive area.	Assumed	This affects high-risk routing and SMO-only or urgent review decisions.		
Gynaecology	Pelvic pain grading likely depends on symptom pattern plus available imaging and prior management.	Gynaecology guideline PDF	Yes, pelvic pain appears to require more contextual judgment than a single rule trigger.	Assumed	This affects whether structured questions alone are sufficient.		
Gynaecology	Urogynaecology, fertility, PCOS, cervical polyp, paediatric gynaecology, pelvic tear, and tubal ligation are all in scope for guideline-based triage.	Gynaecology guideline PDF	Yes, these appear to be intended service categories rather than optional extras.	Assumed	This affects the minimum category coverage for a pilot.		
Workflow	A clinician should remain able to confirm or override any recommendation.	Email plus clinical safety assumption	Yes, clinician control should remain in the loop at least initially.	Assumed	This affects safety, governance, and trust in the workflow.		
Workflow	A coordinator or operational user may need a queue and booking workflow after grading.	Operational assumption from referral grading process	Yes, grading alone is probably not enough without operational handoff.	Assumed	This affects whether the product needs queue, status, and booking support.		
Workflow	Some cases will need return-to-GP, reject, or needs-more-information outcomes rather than a booking.	Gynaecology guideline PDF + service workflow assumption	Yes, not every case should move straight to booking.	Assumed	This affects the shape of the outputs and the user flow after review.		
Pilot	Validation should use redacted real referrals where possible.	Email from Jasveen	Yes, this appears to be how the other vendor is already testing.	Assumed	This affects the fastest path to confirming product fit.		

Area	Requirement or understanding	Source	Current assumption	Status	Why this matters	Please confirm or correct	Notes
Pilot	One service may be piloted before the other if that reduces complexity and risk.	Operational assumption	Yes, colposcopy may be the easier first pilot, but this should be confirmed by the service.	Assumed	This affects project phasing and stakeholder expectation.		

2. Clinical Validation

02-clinical-validation.csv

Service	Category or scenario	Source guideline or document	What the requirement appears to be	Questions or data likely needed	Expected output or priority	Current assumption status	Please confirm or correct	Clinical owner	Priority for pilot	Notes	
COLPOSCOPY	HPV 16 or 18 primary screening referral	COLP Grading Guide	High-priority colposcopy pathway with booking timeframe and category guidance.	HPV result, genotype, cytology, abnormal appearance, prior history, possible registry context.	Likely urgent or high-priority colposcopy output.	Assumed					
COLPOSCOPY	HPV other with suspicious cytology	COLP Grading Guide	Priority depends on combined HPV and cytology pattern.	HPV result, cytology class, prior colposcopy or histology, immune status, appearance.	Priority and clinic pathway based on scenario.	Assumed					
COLPOSCOPY	HPV other with normal or low-grade cytology	COLP Grading Guide	Lower-priority pathway with scenario-specific timing.	HPV result, cytology, prior history, test-of-cure status, registry history.	Routine or lower-priority colposcopy outcome.	Assumed					
COLPOSCOPY	Immune-deficient pathway	COLP Grading Guide	Special handling and possibly different timeframes than standard pathway.	Immune status, HPV result, cytology, prior history, registry history.	Special-case priority and handling.	Assumed					
COLPOSCOPY	Abnormal cervical appearance	COLP Grading Guide + current grading template	Referral may be escalated even if screening results alone are less concerning.	Appearance concern, bleeding, examination note, cytology, HPV, referrer comments.	Potentially urgent pathway.	Assumed					
COLPOSCOPY	Positive test of cure	COLP Grading Guide	Specific re-referral or surveillance logic.	Test-of-cure status, HPV result, cytology, prior treatment history, registry history.	Category-specific follow-up or referral outcome.	Assumed					
COLPOSCOPY	Previous normal colposcopy re-referral	COLP Grading Guide	Registry and prior outcome may alter timing or category.	Prior colposcopy date and result, current HPV/cytology pattern, reason for re-referral.	Scenario-specific timing and category.	Assumed					
COLPOSCOPY	Previous LSIL histology re-referral	COLP Grading Guide	Re-referral may depend on prior histology plus current result.	Prior histology, current HPV/cytology, registry history, elapsed time.	Scenario-specific booking logic.	Assumed					
COLPOSCOPY	Current grading template fields	Current colposcopy grading template image	Operational grading should likely include all fields the current service already uses.	FCT, HPV genotype, investigations, assessment, category, booking priority, clinic, SMO-only, notes, Ovestin, NCSR notes.	Operationally usable colposcopy grading record.	Assumed					
GYNAECOLOGY	AUB	Gynaecology guideline PDF	AUB triage likely depends on USS, severity, age, symptom duration, and management already tried.	Bleeding pattern, USS present, endometrial findings, management tried, anaemia, fibroid context.	Priority	reject	or return-for-more-info decision.	Assumed			
GYNAECOLOGY	PMB	Gynaecology guideline PDF	PMB likely has high-priority logic based on USS and endometrial findings.	PMB episodes, USS presence, endometrial thickness, previous biopsy, age, bleeding recurrence.	Likely P1 or HSC pathway in some scenarios.	Assumed					
GYNAECOLOGY	Fibroids	Gynaecology guideline PDF	Fibroid referrals likely depend on size and symptom burden rather than diagnosis alone.	Fibroid size, bleeding, pressure symptoms, urinary symptoms, USS findings, anaemia.	Priority based on size plus symptoms.	Assumed					
GYNAECOLOGY	Ovarian masses and cysts	Gynaecology guideline PDF	Suspicious adnexal pathology likely requires higher concern routing.	Mass size, solid component, bilateral or complex features, CA-125, hydronephrosis, CT or USS.	Likely P2_HSC or SMO-sensitive pathway in some cases.	Assumed					
GYNAECOLOGY	Pelvic pain	Gynaecology guideline PDF	Pelvic pain probably needs structured symptom and imaging context.	Pain pattern, duration, USS, prior treatment, endometriosis suspicion, acute features.	Priority or reject depending on evidence completeness.	Assumed					
GYNAECOLOGY	Urogynaecology	Gynaecology guideline PDF	Urogynae appears to be scenario-based rather than one-rule triage.	Prolapse, recurrent symptoms, retention, mesh issue, incontinence type, examination findings.	Category-specific priority or return outcome.	Assumed					
GYNAECOLOGY	Fertility	Gynaecology guideline PDF	Fertility may route to a distinct clinic or service rather than standard grading.	Reason for referral, age, duration, prior workup, local policy for fertility referrals.	Distinct pathway	decline	or redirect.	Assumed			
GYNAECOLOGY	PCOS	Gynaecology guideline PDF	PCOS may route to lower-priority or virtual pathway depending on local service rules.	Symptoms, prior management, labs, ultrasound, exclusion of other causes.	Likely lower-priority or alternative pathway.	Assumed					

Service	Category or scenario	Source guideline or document	What the requirement appears to be	Questions or data likely needed	Expected output or priority	Current assumption status	Please confirm or correct	Clinical owner	Priority for pilot	Notes	
GYNAECOLOGY	Cervical polyp	Gynaecology guideline PDF	Management may depend on symptoms, size, and smear history.	Polyp size, bleeding, post-coital bleeding, smear or HPV history, appearance.	Priority or return decision.	Assumed					
GYNAECOLOGY	Paediatric gynaecology	Gynaecology guideline PDF	Paediatric cases may need a distinct pathway and age-sensitive rules.	Age, presenting issue, urgency, safeguarding context, anomaly type.	Special category or specialist pathway.	Assumed					
GYNAECOLOGY	Pelvic tear	Gynaecology guideline PDF	Likely depends on tear grade, symptoms, and chronicity.	Tear grade, pain, continence symptoms, prior repair, urgency.	Priority or specialist review pathway.	Assumed					
GYNAECOLOGY	Tubal ligation	Gynaecology guideline PDF	Likely lower-complexity category but still needs local policy confirmation.	Referral reason, age, parity, counselling requirements, service eligibility.	Routine or policy-defined pathway.	Assumed					
GYNAECOLOGY	Reject or re-refer rules	Gynaecology guideline PDF	Some referrals should not proceed without minimum evidence such as ultrasound or prior management.	Mandatory investigations, minimum referral content, referral completeness rules.	Reject	return to GP	or request-more-info outcome.	Assumed			
GYNAECOLOGY	Virtual clinic or non-bookable routes	Gynaecology guideline PDF	Some categories may be managed without standard booking.	Category, evidence completeness, service preference, SMO review need.	Virtual clinic or non-bookable path.	Assumed					

3. Workflow And Operations

03-workflow-and-operations.csv

Topic	Current understanding	Why this needs confirmation	Questions to answer	Current assumption status	Owner	Mandatory for pilot	Notes
Case creation	A referral becomes a case that is then graded in the system.	We need to know whether cases are created manually, by upload, or from an upstream system.	Who creates the case, and from what source?	Assumed		Yes	
Referral intake	The product may need to accept referral PDFs and supporting documents as a pack.	The intake method affects user workflow and integration design.	Manual upload, inbox, API, or direct source-system feed?	Assumed		Yes	
One-page summary	A summary should be reviewed before grading to reduce clicks.	This appears to be the core practical requirement for gynaecology.	Who reviews it, who edits it, and does it need formal approval?	Assumed		Yes	
Evidence review	Graders may need to see source evidence behind the summary.	Need to confirm whether evidence view is required in pilot or only summary.	Does the grader need quoted evidence and source pages, or only summary text?	Assumed		Maybe	
Clinical recommendation	A system recommendation may be useful once enough information is gathered.	Need to confirm whether pilot should include recommendation or summary only.	Do you want recommendation in phase 1, or summary-only workflow first?	Unknown		Yes	
Clinician override	Clinicians should likely be able to confirm or override the suggested outcome.	This is a major safety and governance assumption.	Who can override, and should override reasons be mandatory?	Assumed		Yes	
SMO review	Some scenarios may need senior review or SMO-only handling.	Need to know which categories or triggers require SMO involvement.	Which cases require SMO review, and is SMO review before or after triage?	Assumed		Yes	
Coordinator workflow	A coordinator may need a queue and booking workflow after grading.	Need to confirm if booking is in scope for pilot or only triage output.	Should the product stop at graded outcome or continue into booking?	Assumed		Maybe	
Return to GP	Some referrals may need to be declined, rejected, or returned for more information.	This materially changes the workflow outputs.	What exact wording and workflow should be used for reject or return-to-GP?	Assumed		Yes	
Needs more information	Some referrals may be incomplete but not rejected.	Need to know whether the service uses a separate status for information requests.	How should missing-information cases be tracked and communicated?	Assumed		Yes	
Printable output	Summary and decision may need to be printed or exported.	Need to know whether print or PDF is operationally required.	What must be printable or exportable for pilot?	Assumed		Maybe	
Internal notes	Some notes may need to stay internal to the service.	Need to distinguish internal notes from outward-facing communication.	Which note types are internal only?	Assumed		Yes	
Referrer-facing notes	Some notes or outcomes may need to be communicated back to GP or referrer.	Need to know whether the product should generate this text or just store it.	What needs to go back to the referrer, and in what format?	Assumed		Maybe	
Statuses and routing	The service likely uses real operational statuses that may differ from generic app statuses.	Need exact service language to avoid building the wrong queue model.	What are the actual statuses, queues, and escalation points used today?	Assumed		Yes	
Booking priority language	Priority may need to be shown as booking timeframe, not just a generic code.	The colposcopy guide and gynae guideline may use different operational language.	How should urgency be presented to users: code, timeframe, clinic type, or all of these?	Assumed		Yes	
Reports	The service may need activity or quality reporting.	Need to know what reports matter for pilot versus later phase.	Which reports are mandatory for pilot: turnaround, backlog, concordance, audit, or other?	Unknown		No	
Pilot scope	The first pilot may need to cover only one service or a subset of categories.	Need to confirm scope to avoid trying to validate everything at once.	Which categories must be in pilot 1, and which can wait?	Unknown		Yes	

4. Technical, Integration, AI, And Hosting

04-technical-integration-ai-hosting.csv

Topic	Question	Current assumption	Why this matters	Possible answers or options	Confirmed answer	Required for pilot	Required for production	Owner	Notes
Hosting	Where must this be hosted?	Likely Health NZ controlled or approved NZ-hosted environment.	This determines whether local, cloud, or third-party options are allowed.	Health NZ-managed environment approved NZ cloud existing internal platform		Yes	Yes		
Hosting	Do you need separate dev, test, and prod environments?	Likely yes for production, but pilot may be lighter.	This affects deployment effort and approval path.	Yes No pilot-only environment first		No	Yes		
Database	Can the product use a dedicated application database?	Likely yes, but not confirmed.	This affects architecture and data ownership.	Dedicated app database existing enterprise database existing clinical platform database		Yes	Yes		
Identity	Is SSO required?	Likely yes if used by Health NZ staff.	This affects authentication design and access management.	Entra ID other SSO local pilot credentials only		No	Yes		
Identity	Is MFA required for privileged users?	Likely yes in production.	This affects user onboarding and governance.	Yes No pilot exception		No	Yes		
Patient data	Where should patient demographics come from?	Unknown.	This affects case creation and identity matching.	PAS EHR referral feed manual entry for pilot		Yes	Yes		
Referrals	Where should referrals come from?	Unknown.	This determines whether the workflow starts from files or upstream data.	API inbox manual upload existing referral platform		Yes	Yes		
Clinic letters	Where should clinic letters come from?	Unknown.	This affects the summary and evidence model.	Document feed EHR manual upload		Yes	Yes		
Discharge summaries	Where should discharge summaries come from?	Unknown.	These were specifically mentioned as part of the evidence set for gynaecology grading.	Document feed EHR manual upload		Maybe	Yes		
Lab results	Where should lab data come from?	Unknown.	Guideline logic may depend on tumour markers or other results.	API HL7 PDF/manual upload not needed for pilot		Maybe	Yes		
Radiology	Where should radiology come from?	Unknown.	Gynaecology grading appears heavily dependent on imaging.	API HL7 PACS/RIS feed PDF/manual upload		Maybe	Yes		
NCSR	Is NCSR mandatory for colposcopy pilot?	Possibly yes for accurate prior-history lookup.	This affects whether colposcopy can be piloted without live restricted integration.	Yes No later phase only for some scenarios		Maybe	Yes		
Restricted access	Who approves access to restricted systems such as NCSR?	Unknown.	This affects real pilot readiness and who can use the system.	Clinical lead information governance digital or integration team		No	Yes		
Integration pattern	What integration method is preferred or available?	Unknown.	This affects how realistic the pilot can be and how much manual fallback is needed.	API FHIR HL7 SFTP document upload only		Maybe	Yes		
Storage	What document storage platform is approved?	Unknown, but likely managed and governed.	This affects file retention, sovereignty, and security.	Health NZ-managed storage Azure Blob other approved storage		Maybe	Yes		
Email or notifications	Is outbound notification or mail delivery available?	Unknown.	This affects reminder or alert features.	SMTP Microsoft relay none in pilot		No	Yes		
AI scope	Is AI assist in scope for the first pilot?	Unknown. Email suggests long-term goal, not necessarily immediate pilot requirement.	This is critical because pilot could be summary-only, deterministic, or AI-assisted.	No AI in pilot deterministic rules only AI summary only AI summary plus recommendation		Yes	Yes		
AI platform	If AI is allowed, which platform is approved?	Unknown.	This determines whether local, Azure OpenAI, or another service is acceptable.	Local model Azure OpenAI other approved service not decided		No	Yes		
Privacy	Can real patient data be used in pilot?	Likely only under approval, otherwise redacted sample cases first.	This affects validation method and system hosting.	Redacted only approved real cases synthetic only		Yes	Yes		
Retention and audit	What audit and data-retention requirements apply?	Unknown.	This affects storage policy, exports, and governance controls.	Short pilot retention enterprise retention audit export required		No	Yes		
Pilot sequencing	Which service should go first?	Colposcopy may be easier, but this needs confirmation.	This is the most important scoping decision for the next phase.	Colposcopy first Gynaecology first both together		Yes	Yes		

5. Decisions And Owners

05-decisions-and-owners.csv

Decision area	Decision needed	Recommended current assumption	Owner needed	Decision by when	Outcome	Notes
Clinical scope	Which service should pilot first?	Colposcopy first, then gynaecology once summary workflow is validated.	Clinical lead			
Clinical scope	Which categories must be in pilot 1?	Start with the highest-volume or highest-pain pathways only.	Clinical lead			
Clinical validation	Who signs off colposcopy workflow and field set?	Named colposcopy clinical owner required.	Colposcopy owner			
Clinical validation	Who signs off gynaecology summary content and grading logic?	Named gynaecology clinical owner required.	Gynaecology owner			
Workflow	Should pilot include booking workflow or stop at triage outcome?	Prefer triage plus visible next action unless booking is explicitly required.	Operational owner			
Workflow	What exact return-to-GP and needs-more-information outcomes are required?	Service language should be confirmed rather than assumed.	Operational owner			
Technical	What hosting environment is approved?	Health NZ controlled or approved NZ-hosted environment.	Technical owner			
Technical	What patient and referral data sources are available in phase 1?	Manual upload plus limited source integration may be sufficient for pilot.	Technical owner			
Technical	Is NCSR required for colposcopy pilot?	Likely needed for some scenarios, but not confirmed.	Technical plus clinical owner			
AI	Is AI in pilot scope at all?	Assume no autonomous grading in pilot; confirm whether any AI assist is wanted.	Clinical plus technical owner			
Governance	Can redacted cases be used for validation and pilot preparation?	Yes, redacted cases are the safest starting point.	Clinical plus governance owner			
Governance	Who will own ongoing validation sessions and decisions?	One owner per stream: colposcopy, gynaecology, workflow, technical.	Programme or project owner			